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NABL
ACCREDITED



Booking ID : 18603655290

Sample Collection Date : 07/Jun/2026

Rashmi Sinha

Female, 48 Yrs

A Comprehensive Health Analysis Report

AI Based Personalized Report for You

Flagship Lab in Gurugram

CAP
ACCREDITED
COLLEGE of AMERICAN PATHOLOGISTS
CAP #: 9019582 | AU-ID: 2107746



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MACHINE & QC DETAILS

LABORATORY EQUIPMENT DASHBOARD

Rashmi sinha

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Siemens Atellica IM 1600

Siemens Healthineers : Siemens Atellica IM 1600

Department - Immunology

No. of test(s) performed - 6

Performed Test(s): CEA-Carcino Embryonic Antigen (Colorectal Cancer Marker Test),Homocysteine,Vitamin B12 Cyanocobalamin,Thyroid Profile-Total (T3, T4 & TSH Ultra-sensitive)

Description: The Siemens Healthineers system manufactured in Germany, combines AI-powered intelligent automation with advanced digitization to enhance performance and workflow efficiency. Utilizing acridinium ester-based chemiluminescence technology, it delivers highly sensitive and specific testing. With a throughput of up to 440 tests per hour and extensive automation, the system reduces manual handling, minimizes human error, and ensures reliable results for laboratories.

Frequency of Quality Control (QC) : 3 times a day (Industry best and as per NABL guidelines)



Beckman AU5800

Beckman Coulter : Beckman AU5800

Department - Biochemistry

No. of test(s) performed - 3

Performed Test(s): Potassium, Serum,Kidney Function Test Advance

Description: The Beckman Coulter clinical chemistry analyzer manufactured in USA, is the fastest in the industry, offering a throughput of 2,000 tests per hour using spectrophotometry and 2900 tests per hour with ISE. It utilizes advanced spectrophotometric technology to ensure high accuracy and reliability, delivering over 21 parameters that meet Six Sigma performance standards.

Frequency of Quality Control (QC) : 3 times a day (Industry best and as per NABL guidelines)



Biorad D100

Bio-Rad : Biorad D100

Department - BIOCHEMISTRY HbA1c

No. of test(s) performed - 2

Performed Test(s): HbA1c

Description: The Bio-Rad fully automated HPLC-based HbA1c analyzer manufactured in USA is a gold-standard system for accurate HbA1c testing. It is NGSP certified, IFCC traceable, and unaffected by common haemoglobin variants, ensuring highly reliable results.

Frequency of Quality Control (QC) : Twice a day (Industry best and as per NABL guidelines)



Horiba Yumi1500

HORIBA : Horiba Yumi1500

Department - Hematology

No. of test(s) performed - 26

Performed Test(s): Complete Blood Count Advance

Description: The fully automated high-end hematology analyzer manufactured in France, features a patented Double Hydrodynamic Sequential System (DHSS) for precise, linear cell flow and enhanced accuracy. It determines total WBC counts using both impedance and flow cytometry technologies. The system provides detailed immature cell analysis—including immature granulocytes, monocytes, lymphocytes, and large immature cells—for improved sepsis screening and cancer pre-screening. Its 360° automated, homogeneous sample mixing minimizes manual intervention and preanalytical errors.

Frequency of Quality Control (QC) : Twice a day (Industry best and as per NABL guidelines)



Horiba Yumi H550E

HORIBA : Horiba Yumi H550E

Department - Hematology

No. of test(s) performed - 1

Performed Test(s): ESR Automated

Description: The fully automated DIFF + ESR analyzer manufactured in France, utilizes i-Double Diff and CORA ESR technologies, with CORA providing ESR results through advanced Correlated Rouleaux Analysis of RBC aggregation for precise and reliable hematology testing.

Frequency of Quality Control (QC) : Twice a day (Industry best and as per NABL guidelines)

HEALTH ANALYSIS

Personalized Summary & Vital Parameters

Rashmi Sinha

Booking ID : 18603655290 | Sample Collection Date : 07/Jun/2026

Rashmi Sinha,

Congratulations, We have successfully completed your health diagnosis. This is a big step towards staying on top of your health and identify potential to improve!

10 Vital Health Parameters of a Human Body Ecosystem

Below are the health parameters which require routine checkups for primary healthcare. The view also includes *personalised information* depending on the tests you have taken.

Your Health Score

73

Out of 100

*Calculated from test reports



Thyroid Function

Thyroid Stimulating Hormone (TSH) – Ultrasensitive : 18.42 μ IU/ml

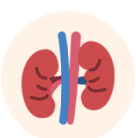
● **Concern**



Cholesterol Total

194 mg/dl

● **Everything looks good**



Kidney Function

Serum Creatinine : 0.37 mg/dl

● **Concern**



Vitamin D

4.26 ng/ml

● **Concern**



HbA1c

5.1 %

● **Everything looks good**



Vitamin B12

161 pg/ml

● **Concern**



Liver Function

Alanine Aminotransferase (ALT/SGPT) : 13.5 U/L

● **Everything looks good**



Calcium Total

8.8 mg/dl

● **Everything looks good**



Iron studies

Serum Iron : 83 μ g/dl

● **Everything looks good**



Complete Hemogram

Haemoglobin (HB) : 12.5 g/dL

● **Everything looks good**



HEALTH ANALYSIS

Critical Parameters

Rashmi Sinha

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We have observed that the below given critical parameters have shown out of range results, which can have negative impact on your health.

Creatinine, Serum

Creatinine is a chemical waste in your blood, produced from muscle metabolism and excess meat consumption. It is normally removed from your blood by your kidneys, but when kidney function slows down, the creatinine level rises. The Creatinine Serum test is hence required to monitor kidney functions.

Impact on overall health?

This test assess your kidney function, determines your risk of kidney damage and renal complications of high blood pressure or diabetes.

How to improve health conditions?

In case of high creatinine levels, consult a doctor for clinical evaluation and discuss further tests. It is often advisable to reduce protein intake and avoid strenuous exercises.

Your Result Value

↓ **0.37** mg/dl

Concern

Normal Value

● **0.4-0.9 mg/dl**

WBC-Total Counts Leucocytes

WBC total measures the count of white blood cells (WBC) in blood. This test is used to screen or diagnose conditions which influence the WBC count. It helps diagnose infection or inflammatory conditions.

Impact on overall health?

High values indicate infection, or inflammation or allergies. An abnormally high count is seen in severe infections, leukaemia and decreased levels in bone marrow depression.

How to improve health conditions?

If your WBC count is high or low, consult your physician for clinical evaluation, other tests and treatment. Lifestyle modifications like exercise and avoiding smoking can help.

Your Result Value

↓ **3.7** $10^3/\mu\text{L}$

Concern

Normal Value

● **4-10 $10^3/\mu\text{L}$**

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Referred By	: Self	Report Generated On	: 07/Jun/2026 08:01PM	
Customer Since	: 07/Jun/2026	Sample Temperature	: Maintained ✓	
Sample Type	: Whole Blood EDTA	Report Status	: Final Report	

DEPARTMENT OF BIOCHEMISTRY HBA1C

Healthians Summer Wellness Package Female FOR COUPLE COMBO

Test Name	Value	Unit	Bio. Ref Interval
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HbA1c - Glycosylated Hemoglobin

Hba1c (Glycosylated Hemoglobin)	5.10	%	4.2 - 5.7
Method: HPLC			
Machine: BIORAD D100			
Average Estimated Glucose	99.67	mg/dl	
Method: Calculated			

INTERPRETATION:

AS PER AMERICAN DIABETES ASSOCIATION (ADA):

REFERENCE GROUP

Non diabetic
At Risk (Prediabetes)
Diagnosing Diabetes

GLYCOSYLATED HEMOGLOBIN (HBA1c) in %

<5.7
5.7 – 6.4
≥ 6.5
Age > 19 Years
Goals of Therapy: < 7.0
Actions Suggested: >8.0
Age < 19 Years
Goal of therapy: <7.5

Therapeutic goals for glycemic control

REMARKS

- HbA1c is used for monitoring diabetic control. It reflects the mean plasma glucose over three months
- HbA1c may be falsely low in diabetics with hemolytic disease. In these individuals a plasma fructosamine level may be used which evaluates diabetes over 15 days.
- Inappropriately low HbA1c values may be reported due to hemolysis, recent blood transfusion, acute blood loss, hypertriglyceridemia, chronic liver disease. Drugs like dapson, ribavirin, antiretroviral drugs, trimethoprim, may also cause interference with estimation of HbA1c, causing falsely low values.
- HbA1c may be increased in patients with polycythemia or post-splenectomy.
- Inappropriately higher values of HbA1c may be caused due to iron deficiency, vitamin B12 deficiency, alcohol intake, uremia, hyperbilirubinemia and large doses of aspirin.
- Trends in HbA1c are a better indicator of diabetic control than a solitary test. 7. Any sample with >15% HbA1c should be suspected of having a hemoglobin variant, especially in a non-diabetic patient. Similarly, below 4% should prompt additional studies to determine the possible presence of variant hemoglobin.
- HbA1c target in pregnancy is to attain level <6 % .
- HbA1c target in paediatric age group is to attain level < 7.5 %.
- Method : Ion-exchange high-performance liquid chromatography (HPLC).
- Reference : American Diabetes Associations. Standards of Medical Care in Diabetes 2023
- The presence of some hemoglobinopathies/hemoglobin variants may be missed by some platforms used for measuring HbA1c values. In these cases, the HbA1c result may vary or give deviant results, depending on the platform used. Alternative measures of glycemic control like fructosamine or glycated albumin may be considered in such cases.
- In presence of hemoglobinopathy/hemoglobin variants in homozygous/compound heterozygous conditions alternative forms of testing such as fructosamine or glycated albumin may be considered.

Pranjali

Dr. Pranjali Sejwal
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Consultant Pathologist
Reg. No.: H N-20567, Healthians Labs



SIN No:E8267988

Healthians Labs (A Unit of Expedient Healthcare Marketing Pvt. Ltd.)

Plot 1 & 2, Udyog Vihar, Phase-4, Gurgaon, Haryana, Pincode-122016 (CAP Number: 9019582 | NABL Accreditation Certificate Number MC-4245)

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Referred By	: Self	Report Generated On	: 07/Jun/2026 06:38PM	
Customer Since	: 07/Jun/2026	Sample Temperature	: Maintained ✓	
Sample Type	: Sodium Fluoride	Report Status	: Final Report	

DEPARTMENT OF BIOCHEMISTRY

Healthians Summer Wellness Package Female FOR COUPLE COMBO

Test Name	Value	Unit	Bio. Ref Interval
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Fasting Blood Sugar

Glucose, Fasting	80.3	mg/dl	70 - 100
Method: Hexokinase			
Machine: BECKMAN COULTER DXC 700 AU			

American Diabetes Association Reference Range :

Normal	: < 100 mg/dl
Impaired fasting glucose(Prediabetes)	: 100 - 125 mg/dl
Diabetes	: >= 126 mg/dl

Conditions that can result in an elevated blood glucose level include:

- Diabetes mellitus ,Hemochromatosis ,Cushing syndrome ,Acromegaly and gigantism.
- Increased circulating epinephrine such as in pheochromocytoma and adrenal injections
- Acute pancreatitis
- Chronic pancreatitis

Conditions that cause low blood glucose level include :

- Pancreatic disorders : Islet cell tumor , pancreatitis
- Hepatic disease (diffuse severe disease)
- Endocrine disorders : hypopituitarism, Addison's disease ,hypothyroidism
- Alcoholism
- Malnutrition



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Consultant Biochemist
DMC - 97534, Healthians Labs



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Referred By	: Self	Report Generated On	: 07/Jun/2026 08:10PM	
Customer Since	: 07/Jun/2026	Sample Temperature	: Maintained ✓	
Sample Type	: Serum	Report Status	: Final Report	

DEPARTMENT OF BIOCHEMISTRY

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Test Name	Value	Unit	Bio. Ref Interval
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Potassium - Serum

Serum Potassium	4.81	mmol/L	3.5 - 5.5
Method: ISE (Indirect)			
Machine: BECKMAN COULTER AU 5800			

Potassium is a primary intracellular ion, only 2 % is extracellular, high intracellular concentration is maintained by a Na- K ATPase pump, which continuously transports potassium into the cell against a concentration gradient. The pump is a critical factor in maintaining and adjusting the ionic gradients, on which nerve impulse transmission and contractility of cardiac and skeletal muscle depends. In acidemia, potassium moves out of the cells, in alkalemia, potassium moves into the cells. Hypokalemia inhibits aldosterone production and hyperkalemia stimulates aldosterone production.

Creatine Kinase(CPK) - Serum

CK CREATINE KINASE - (CPK)	65	U/L	<145
Method: NAC activated			
Machine: BECKMAN COULTER AU 5800			

Measurements of CK are primarily used in the diagnosis and treatment of myocardial infarction as well as being the most sensitive indicator of muscle damage. It is diagnosis and monitoring of myopathies and cerebral vascular disease. It increased in Myocardial infarction (MI), myocarditis, muscle trauma, rhabdomyolysis, muscular dystrophy, polymyositis, severe muscular exertion, malignant hyperthermia, hypothyroidism, cerebral infarction, surgery, Reye syndrome, tetanus, generalized convulsions, alcoholism, IM injections, DC counter shock, Drugs:clotfibrate, HMG-CoA reductase inhibitors. During an MI, serum CK level rises rapidly (within 3-5 hours); reaches a maximum after 12 - 14 hrs and returns to normal range after 3-4 days post-MI. Total CK is not specific enough for use in diagnosis of MI, but a normal total CK has a high negative predictive value. A more specific test is needed for diagnosis of MI or acute coronary syndrome (eg, CK-MB, Troponin T and Troponin I)



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Test Name	Value	Unit	Bio. Ref Interval
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Rheumatoid Factor (RA) - Quantitative - Serum

RHEUMATOID FACTOR	<10	IU/mL	<14
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Method: Latex Particle Immunoturbidimetric

Machine: BECKMAN COULTER AU 5800

Rheumatoid factor is an immunoglobulin present in serum of 50 -95% of adults with Rheumatoid arthritis(RA).It appears in serum and synovial fluid several months after onset of RA and is present upto years after therapy

Use

- Assisting in the diagnosis of RA ,especially when clinical diagnosis is difficult

Increased in : RA ,Chronic hepatitis , chronic viral infection ,Cirrhosis, SLE, Scleroderma ,syphilis ,Infectious mononucleosis, TB ,Sjogren syndrome , Sarcoidosis , malaria, Leishmaniasis

Amylase - Serum

AMYLASE	47	U/L	22 - 80
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Method: CNPG3

Machine: BECKMAN COULTER AU 5800

Amylase is produced by exocrine pancreas and also by the salivary glands. it is used to evaluate pancreatic function and also used in the diagnosis and management of pancreatitis.

Diseases resulting in elevation of plasma alpha-amylase include: acute pancreatitis, parotitis, alcoholism, renal insufficiency and diseases such as viral hepatitis, AIDS, abdominal typhoid, sarcoidosis and trauma to the upper abdomen. There is also a detectable increase in amylase after an ERCP procedure. In acute pancreatitis, amylase increases 5-6 hours after the onset of symptoms and remains elevated for 2-5 days. The increase in plasma activity does not reflect disease severity and conversely, extensive destruction of the pancreas may not cause a significant increase in the plasma concentration of pancreatic alpha-amylase.

Lipase - Serum

LIPASE	22	U/L	<67
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Method: Colorimetric

Machine: BECKMAN COULTER AU 5800

Lipase is produced in the acinar cells of the pancreas and is responsible for the hydrolysis of water-insoluble long chain fatty acid esters of glycerol. Lipase measurement in serum and plasma is used exclusively for the investigation of pancreatic disorders, usually pancreatitis. Serum lipase may be elevated in acute pancreatitis, acute episodes of chronic pancreatitis and obstructive pancreatitis

Slight elevations are also frequently present in diabetic ketoacidosis, viral hepatitis, epidemic parotiditis, abdominal typhoid and sarcoidosis, due to involvement of the pancreas.

Preeti

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DEPARTMENT OF BIOCHEMISTRY

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Test Name	Value	Unit	Bio. Ref Interval
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hs CRP (C-Reactive Protein high sensitive)

HS-CRP (HIGH SENSITIVITY C-REACTIVE PROTEIN)	0.35	mg/L	< 1.0
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Method: Latex Particle Immunoturbidimetry

Machine: BECKMAN COULTER AU 5800

High sensitivity C-reactive protein, when used in conjunction with other clinical laboratory evaluation of acute coronary syndromes, may be useful as an independent marker of prognosis for recurrent events in patients with stable coronary disease or acute coronary syndrome. Hs-CRP levels should not be substituted for assessment of traditional cardiovascular risk factors. Patients with persistently unexplained, marked elevation of hs-CRP after repeated testing should be evaluated for non-cardiovascular etiologies.

Clinical significance :

Hs-CRP measurements may be used as an independent risk marker for the identification of individuals at risk for future cardiovascular disease. Elevated CRP values may be indicative of prognosis of individuals with acute coronary syndromes, and may be useful in the management of such individuals.



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Sample Type	: Serum	Report Status	: Final Report	

DEPARTMENT OF BIOCHEMISTRY

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Test Name	Value	Unit	Bio. Ref Interval
Liver Function Test (LFT)			
Serum Bilirubin, (Total)	0.62	mg/dl	0.3 - 1.2
Method: DPD			
Machine: BECKMAN COULTER AU 5800			
Serum Bilirubin, (Direct)	0.09	mg/dl	0 - 0.2
Method: DPD			
Machine: BECKMAN COULTER AU 5800			
Serum Bilirubin, (Indirect)	0.53	mg/dl	0.0 - 0.8
Method: Calculated			
Aspartate Aminotransferase (AST/SGOT)	20.00	U/L	3 - 35
Method: UV without P5P			
Machine: BECKMAN COULTER AU 5800			
Alanine Aminotransferase (ALT/SGPT)	13.5	U/L	3 - 35
Method: UV without P5P			
Machine: BECKMAN COULTER AU 5800			
Alkaline Phosphatase (ALP)	86.00	U/L	33-98
Method: AMP Buffer			
Machine: BECKMAN COULTER AU 5800			
Gamma Glutamyl Transferase (GGT)	13.0	U/L	5- 38
Method: IFCC			
Machine: BECKMAN COULTER AU 5800			
Serum Total Protein	7.07	gm/dl	6.6 - 8.3
Method: BIURET			
Machine: BECKMAN COULTER AU 5800			
Serum Albumin	4.29	g/dl	3.5 - 5.2
Method: BROMOCRESOL GREEN			
Machine: BECKMAN COULTER AU 5800			
Serum Globulin	2.78	gm/dl	3.0 - 4.2
Method: Calculated			
Albumin/Globulin Ratio	1.54	Ratio	1.2 - 2.5
Method: Calculated			
SGOT/SGPT Ratio	1.48	Ratio	0.7 - 1.4
Method: Calculated			

Bilirubin is a yellowish pigment found in bile and is a breakdown product of normal heme catabolism. Elevated levels are a result of increased bilirubin production (e.g hemolysis and ineffective erythropoiesis), decreased bilirubin excretion (e.g.; obstruction and hepatitis) and abnormal



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Test Name	Value	Unit	Bio. Ref Interval
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bilirubin metabolism (e.g; hereditary and neonatal jaundice) .

Conjugated (direct) bilirubin is elevated in conditions like- Hereditary disorders(Dubin Johnson syndrome, Rotor syndrome),Hepatocellular damage(e.g –viral ,toxic ,alcohol ,drugs) ,biliary duct obstruction (extrahepatic or intrahepatic), Infiltrations ,space occupying lesions(e.g metastasis, abscess , granuloma , amyloidosis. Increased unconjugated (indirect) bilirubin may be a result of hemolytic or pernicious anemia, transfusion reaction & a common metabolic condition termed Gilbert syndrome.

AST levels increase in viral hepatitis, blockage of the bile duct ,cirrhosis of the liver, liver cancer, kidney failure, hemolytic anemia, pancreatitis, hemochromatosis. AST levels may also increase after a heart attack or strenuous activity.

ALT is a liver specific enzyme commonly measured as a part of a diagnostic evaluation of hepatocellular injury, to determine liver health.

Elevated ALP levels are seen in Biliary Obstruction, Osteoblastic Bone Tumors, Osteomalacia, Hepatitis, Hyperparathyroidism, Leukemia, Lymphoma, Paget's disease, Rickets, Sarcoidosis etc.

Elevated serum GGT activity can be found in diseases of the liver, Biliary system and pancreas. Obstructive liver disease, high alcohol consumption and use of enzyme-inducing drugs lead to raised GGT levels .

Serum total protein measures the total amount of protein in serum. It is largely comprised of albumin and globulins. Increased levels may be due to: Chronic inflammation or infection, including HIV and hepatitis B or C, multiple myeloma, Waldenstrom's disease. Decreased levels may be due to: Agammaglobulinemia, Bleeding (hemorrhage), Burns, Glomerulonephritis, Liver disease, Malabsorption, Malnutrition, Nephrotic Syndrome.

Albumin is the most abundant protein in the serum and is produced in the liver. Low serum albumin levels (hypoalbuminemia) can be caused by: Liver diseases like liver cirrhosis, nephrotic syndrome, protein-losing enteropathy, burns, hemodilution, increased vascular permeability or decreased lymphatic clearance, malnutrition and wasting .

Globulins are increased in most liver diseases , in chronic inflammatory diseases and neoplastic diseases.

Please note that there is a change in the Biological Reference Interval for Alkaline Phosphatase (ALP) parameter. (Dated: 11.02.2026).



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Customer Since	: 07/Jun/2026	Sample Temperature	: Maintained ✓	
Sample Type	: SERUM	Report Status	: Final Report	

DEPARTMENT OF BIOCHEMISTRY

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Test Name	Value	Unit	Bio. Ref Interval
Iron study			
Serum Iron	83.0	ug/dl	60 - 180
Method: TPTZ			
Machine: BECKMAN COULTER AU 5800			
UIBC	308.00	ug/dl	155 - 355
Method: Nitroso-PSAP			
Machine: BECKMAN COULTER AU 5800			
Serum Total Iron Binding Capacity (TIBC)	391	µg/dl	250 - 400
Method: FE+UIBC (saturation with iron)			
Transferrin Saturation %	21.23	%	15 - 50
Method: Calculated			

Iron participates in a variety of vital processes in the body varying from cellular oxidative mechanisms to the transport and delivery of oxygen to body cells. It is a constituent of the oxygen-carrying chromoproteins, haemoglobin and myoglobin, as well as various enzymes, such as cytochrome oxidase and peroxidases.

Serum iron may be increased in hemolytic, megaloblastic and aplastic anemias, and in hemochromatosis acute leukemia, lead poisoning, pyridoxine deficiency, thalassemia, excessive iron therapy, and after repeated transfusions. Drugs causing increased serum iron include chloramphenicol, cisplatin, estrogens (including oral contraceptives), ethanol, iron dextran, and methotrexate. Iron can be decreased in iron-deficiency anemia, acute and chronic infections, carcinoma, nephrotic syndrome hypothyroidism, in protein- calorie malnutrition and after surgery. Diurnal variation is seen in serum iron levels with normal values obtained in the midmorning, low values in midafternoon and very low values near midnight.

TIBC measures the blood's capacity to bind iron with transferrin (TRF). Estrogens and oral contraceptives increase TIBC levels. Asparaginase, chloramphenicol, corticotropin, cortisone, and testosterone decrease the TIBC levels.

Transferrin is the primary plasma iron transport protein, which binds iron strongly at physiological pH. Transferrin is generally only 25% to 30% saturated with iron. The additional amount of iron that can be bound is the unsaturated iron-binding capacity (UIBC). Transferrin saturation represents the number of iron-binding sites that are occupied. It is a better index of iron stores than serum iron alone. Transferrin saturation is decreased in iron deficiency anemia (usually <10% in established deficiency).



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Customer Since	: 07/Jun/2026	Sample Temperature	: Maintained ✓	
Sample Type	: Serum	Report Status	: Final Report	

DEPARTMENT OF BIOCHEMISTRY

Healthians Summer Wellness Package Female FOR COUPLE COMBO

Test Name	Value	Unit	Bio. Ref Interval
Kidney Function Test Advance			
Serum Creatinine Method: Modified Jaffe, Kinetic Machine: BECKMAN COULTER AU 5800	0.37	mg/dl	0.4 - 0.9
GFR, ESTIMATED Method: Calculated (CKD-EPI)	124.32	mL/min/1.73m2	
Serum Uric Acid Method: Uricase PAP Machine: BECKMAN COULTER AU 5800	2.8	mg/dl	2.6-6.0
Serum Calcium Method: ARSENAZO III Machine: BECKMAN COULTER AU 5800	8.8	mg/dl	8.8 - 10.6
Serum Phosphorus Method: PHOSPHOMOLYBDATE COMPLEX Machine: BECKMAN COULTER AU 5800	3.6	mg/dl	2.5 - 4.5
Serum Sodium Method: ISE (Indirect) Machine: BECKMAN COULTER AU 5800	140	mmol/L	136 - 146
Serum Chloride Method: ISE (Indirect) Machine: BECKMAN COULTER AU 5800	105	mmol/L	101 - 109
Blood Urea Method: GLDH,Kinetic assay Machine: BECKMAN COULTER AU 5800	16.7	mg/dl	17 - 43
Blood Urea Nitrogen (BUN) Method: Calculated	7.8	mg/dl	8 - 20
Bun/Creatinine Ratio Method: Calculated	21.08	Ratio	
Urea/Creatinine Ratio Method: Calculated	45.14	Ratio	

The kidneys play a vital role in the excretion of waste products and toxins such as urea, creatinine and uric acid, regulation of extracellular fluid volume, serum osmolality and electrolyte concentrations, as well as the production of hormones like erythropoietin and 1,25 dihydroxy vitamin D and renin. Assessment of renal function is important in the management of patients with kidney disease or pathologies affecting renal function. Tests of renal function have utility in identifying the presence of renal disease, monitoring the response of kidneys to treatment, and determining the progression of renal disease.

Urea is synthesized in the liver as the final product of protein and amino acid metabolism. Urea synthesis is therefore dependent on daily protein intake and endogenous protein metabolism.



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SIN No:E8267988

Patient Name	: Rashmi Sinha	Barcode	: E8267988	
Age/Gender	: 48Y 0M 0D /Female	Sample Collected On	: 07/Jun/2026 07:01AM	
Order Id	: 18603655290	Sample Received On	: 07/Jun/2026 05:49PM	
Referred By	: Self	Report Generated On	: 07/Jun/2026 08:24PM	
Customer Since	: 07/Jun/2026	Sample Temperature	: Maintained ✓	
Sample Type	: Serum	Report Status	: Final Report	

DEPARTMENT OF BIOCHEMISTRY

Healthians Summer Wellness Package Female FOR COUPLE COMBO

Test Name	Value	Unit	Bio. Ref Interval
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Creatinine is a metabolic product of creatine and phosphocreatine, which are both found almost exclusively in muscle. Uric Acid is the major product of purine catabolism in humans. Uric acid levels are used to monitor the treatment of gout. Measurement of calcium is used in the diagnosis and treatment of parathyroid disease, a variety of bone diseases, chronic renal disease, urolithiasis and tetany. Phosphorus levels are increased in acute or chronic renal failure with decreased GFR. Sodium is an electrolyte, and it helps regulate the amount of water in and around the cells & the balance of chemicals in the body called acids and bases. Chloride is a negatively charged ion that works with other electrolytes such as potassium, sodium, and bicarbonate, to help regulate the amount of fluid in the body and maintain the acid-base balance.

EGFR-National kidney Disease Education program recommends the use of MDRD equation to predict/ estimate GFR in adults (≥ 20 years) with Chronic kidney disease. Calculated by serum creatinine may be less accurate due to certain factor like race muscle mass, diet and certain drugs in such case EGFR should be calculated using serum cystatin C.

Age (Years)	Male ML/min/1.73m ²	Female ML/min/1.73m ²
17-24	93-131	—
25-34	78-146	—
35-44	74-138	65-123
45-54	74-129	58-110
55-64	69-122	50-110
65-74	61-114	46-105
75-84	52-102	48-85



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DEPARTMENT OF BIOCHEMISTRY

Healthians Summer Wellness Package Female FOR COUPLE COMBO

Test Name	Value	Unit	Bio. Ref Interval
Lipid Profile			
Total Cholesterol Method: CHO-POD (Trac. Abel-Kendall) Machine: BECKMAN COULTER AU 5800	194.0	mg/dl	Desirable : <200 Borderline: 200-239 High : >=240
Serum Triglycerides Method: GPO-POD Machine: BECKMAN COULTER AU 5800	75.0	mg/dL	Desirable : <150 Borderline high : 150-199 High : 200-499 Very high : > 500
Serum HDL Cholesterol Method: ENZYMATIC IMMUNOINHIBITION Machine: BECKMAN COULTER AU 5800	66.0	mg/dL	40 - 60
LDL Cholesterol Calculated Method: Calculated	113.00	mg/dL	Optimal : <100 near /above Optimal:100 - 129 Borderline High: 130- 159 High : 160 - 189 Very High :>=190
VLDL Cholesterol Calculated Method: Calculated	15	mg/dl	<30
Total CHOL / HDL Cholesterol Ratio Method: Calculated	2.94	Ratio	3.30 - 4.40
LDL / HDL Cholesterol Ratio Method: Calculated	1.71	Ratio	Desirable/Low Risk: 0.5-3.0 Line/Moderate Risk: 3.0-6.0 Elevated/High Risk: >6.0
HDL / LDL Cholesterol Ratio Method: Calculated	0.58	Ratio	Optimal->0.4 Moderate-0.4 to 0.3 High-<0.3
Non-HDL Cholesterol Method: Calculated	128.0	mg/dL	0.0 - 160.0

Dyslipidemia is a disorder of fat or lipoprotein metabolism in the body and includes lipoprotein overproduction or deficiency. Dyslipidemias means increase in the level of one or more of the following: Total Cholesterol, low density lipoprotein (LDL) and/or triglyceride concentrations. Dyslipidemia also includes a decrease in the "good" cholesterol or high-density lipoprotein (HDL) concentration in the blood. Cholesterol is a steroid carried in the bloodstream as lipoprotein, necessary for cell membrane functioning and as a precursor to bile acids, progesterone, vitamin D, estrogens, glucocorticoids and mineralocorticoids.



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DEPARTMENT OF BIOCHEMISTRY

Healthians Summer Wellness Package Female FOR COUPLE COMBO

Test Name	Value	Unit	Bio. Ref Interval
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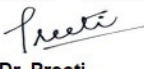
HDL is termed "good cholesterol" because its levels are inversely related to the risk of Coronary heart disease.

LDL cholesterol is termed the "bad cholesterol" and their increased levels are associated with increased risk of atherosclerosis and coronary heart disease.

Lipid level assessments must be made following 9 to 12 hours of fasting, otherwise assay results might lead to erroneous interpretation. Healthians labs report biological reference intervals (normal ranges) in accordance with the recommendations of The National Cholesterol Education Program (NCEP) & Adult Treatment Panel IV (ATP IV) guidelines providing the most desirable targets of various circulating lipid fractions in the blood. NCEP recommends that all adults above 20 years of age must be screened for abnormal lipid levels.

Updated risk stratification approach recommended by the Lipid Association of India (2023)			
Risk Category			
Extreme Risk	Category A: ASCVD with ≥ 1 feature of high-risk group ●CACS ≥300 ●Homozygous FH		
	Category B: ASCVD with ≥ 1 feature of very high-risk group or ● Recurrent ACS or ● Polyvascular disease or ●Homozygous FH		
	Category C: Recurrent ASCVD event despite LDL-C around 30 mg/dL. These patients require special consideration.		
Very High Risk (VHR)	●Diabetes with target organ damage ● Diabetes with ≥2 major ASCVD risk factors ●CACS 100–299 or >75 percentile if CACS 1–99 ●≥2 high risk features ●Established ASCVD (obstructive or non-obstructive) ●Heterozygous FH or LDL-C ≥190 mg/dL		
High Risk (HR)	●>3 major ASCVD risk factors, or ●LDL-C 160–189 mg/dL or ●Non-HDL-C 190–219 mg/dL or ●Diabetes with 0–1 major ASCVD risk factor ●2 major ASCVD factors + ≥1 risk modifier or ●Any 1 high-risk feature.		
Moderate Risk (MR)	●2 major ASCVD risk factors, or ●LDL-C 130–159 mg/dL or ●Non-HDL-C 160–189 mg/dL or ●Low-risk group + ≥1 risk modifier or lifetime ASCVD risk >30%		
Low Risk (LR)	●0–1 major ASCVD risk factor and ●LDL-C 100–129 mg/dL and ●Non-HDL-C 130–159 mg/dL, and ●Lifetime CVD risk <30%		
Major Risk ASCVD (Atherosclerotic Cardiovascular Disease) Risk Factors			
●Age ≥45 years in males and ≥55 years in females ●Current cigarette smoking or tobacco use ●High blood pressure ●Low HDL-C			
Treatment targets for lipid lowering therapy for various atherosclerotic cardiovascular disease risk groups			
Risk Groups	Treatment Targets		
	LDL-C, mg/dl (Primary Target)	Non-HDL-C,mg/dl(Coprimary Target)	Apo-B, mg/dl (Secondary Target)
Low risk group	<100	<130	<90
Moderate risk group	<100 (optional <70)	<130 (optional <100)	<90
High risk group	<70	<100	<80
Very high risk	<50	<80	<65
Extreme -risk-group- Category (A)	<50 (optional ≤30)	<80 (optional ≤60)	<65
Extreme -risk-group- Category (B)	≤30	≤60	<50
Extreme -risk-group- Category (C)	10-15	40-45	-----

Reference: Journal of Clinical Lipidology, Vol-18 No-3. May/June 2024


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Customer Since	: 07/Jun/2026	Sample Temperature	: Maintained ✓	
Sample Type	: Urine	Report Status	: Final Report	

DEPARTMENT OF CLINICAL PATHOLOGY

Healthians Summer Wellness Package Female FOR COUPLE COMBO

Test Name	Value	Unit	Bio. Ref Interval
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Urine Routine & Microscopy Extended

PHYSICAL EXAMINATION

Colour	Yellow		Pale Yellow
Method: Visual			
Volume	15.00	mL	
Method: Visual			
Appearance	Clear		Clear
Method: Visual			

CHEMICAL EXAMINATION

Specific Gravity	1.015		1.001 - 1.035
Method: Dipstick-Ion exchange			
Machine: URIPLUS_600			
pH	8.0		4.5 - 7.5
Method: Dipstick-Double indicator			
Machine: URIPLUS_600			
Glucose	Negative		Negative
Method: Dipstick-oxidase peroxidase			
Machine: URIPLUS_600			
Urine Protein	Negative		Negative
Method: Dipstick-Bromophenol blue			
Machine: URIPLUS_600			
Ketones	Negative		Negative
Method: Sodium nitroprusside			
Machine: URIPLUS_600			
Urobilinogen	Normal		Normal
Method: Dipstick-Ehrlichs Test			
Machine: URIPLUS_600			
Bilirubin	Negative		Negative
Method: Dipstick-Ehrlichs Test			
Machine: URIPLUS_600			
Nitrite	Negative		Negative
Method: Dipstick-Griess test			
Machine: URIPLUS_600			
Blood	Negative		Nil

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Plot 1 & 2, Udyog Vihar, Phase-4, Gurgaon, Haryana, Pincode-122016 (CAP Number: 9019582 | NABL Accreditation Certificate Number MC-4245)

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Customer Since	: 07/Jun/2026	Sample Temperature	: Maintained ✓	
Sample Type	: Urine	Report Status	: Final Report	

DEPARTMENT OF CLINICAL PATHOLOGY

Healthians Summer Wellness Package Female FOR COUPLE COMBO

Test Name	Value	Unit	Bio. Ref Interval
Method: Dipstick-Peroxidase Machine: URIPLUS_600			
Leucocyte Esterase	Negative		Nil
Method: Dipstick- Esterase Machine: URIPLUS_600			
MICROSCOPIC EXAMINATION			
Pus Cells	1-2	/HPF	0 - 5
Method: Microscopic Examination			
Epithelial cells	1-2	/HPF	0 - 5
Method: Microscopic Examination			
RBCs	Nil	/HPF	Nil
Method: Microscopic Examination			
Casts	Nil		Nil
Method: Microscopic Examination			
Crystals	Nil		Nil
Method: Microscopic Examination			
Bacteria	Absent		Absent
Method: Microscopic Examination			
Yeast Cell	Absent		Absent
Method: Microscopic Examination			
Others (Non Specific)	Nil		NIL
Method: Microscopic Examination			

The main indication for testing for glucose in urine is detection of unsuspected diabetes mellitus or follow-up of known diabetic patients. Renal glycosuria accounts for 5% of cases of glycosuria in general population.

Proteinuria can be seen in nephrotic syndrome, pyelonephritis, heavy metal poisoning, tuberculosis of kidney, interstitial nephritis, cystinosis, Fanconi syndrome, rejection of kidney transplant. Hemodynamic proteinuria is transient and can be seen in high fever, hypertension, heavy exercise, congestive cardiac failure, seizures, and exposure to cold. Post-renal proteinuria is caused by inflammatory or neoplastic conditions in renal pelvis, ureter, bladder, prostate, or urethra.

Ketonuria can be seen in uncontrolled Diabetes mellitus with ketoacidosis, Glycogen storage disorder, starvation, persistent vomiting in children, weight reduction program, fever in children, severe thyrotoxicosis, pregnancy and protein calorie malnutrition.

Presence of bilirubin in urine indicates conjugated hyperbilirubinemia (obstructive or hepatocellular jaundice). Bile salts along with bilirubin can be detected in urine in cases of obstructive jaundice. Normally about 0.5-4 mg of urobilinogen is excreted in urine in 24 hours. Therefore, a small amount of urobilinogen is normally detectable in urine. Increased urobilinogen in urine can be seen due to hemolysis, megaloblastic anemia and haemorrhage in tissues. Decreased urobilinogen can be seen in obstructive jaundice, reduction of intestinal bacterial flora, neonates and following antibiotic treatment. The presence of abnormal number of intact red blood cells in urine is called as hematuria. It implies presence of a bleeding lesion in the urinary tract. Hematuria can be seen in glomerular diseases like Glomerulonephritis, Berger's disease, lupus nephritis, Henoch-Schonlein purpura, non glomerular diseases like Calculus, tumor, infection, tuberculosis, pyelonephritis, hydronephrosis, polycystic kidney disease, trauma, after strenuous physical exercise, diseases of prostate (benign hyperplasia of prostate, carcinoma of prostate).

Nitrites are not present in normal urine. Ingested nitrites are converted to nitrate and excreted

in urine. If gram-negative bacteria (e.g. E.coli, Salmonella, Proteus, Klebsiella, etc.) are present in urine, they will reduce the nitrates to nitrites through the action of bacterial enzyme nitrate reductase. As E. coli is the commonest organism causing urinary tract infection, this test is helpful as a screening test for urinary tract

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Customer Since	: 07/Jun/2026	Sample Temperature	: Maintained ✓	
Sample Type	: Urine	Report Status	: Final Report	

DEPARTMENT OF CLINICAL PATHOLOGY

Healthians Summer Wellness Package Female FOR COUPLE COMBO

Test Name	Value	Unit	Bio. Ref Interval
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infection.

Some organisms like Staphylococci or Pseudomonas do not reduce nitrate to nitrite and therefore in such infections nitrite test is negative.

Leucocyte esterase test detects esterase enzyme released in urine from granules of leucocytes. Thus the test is positive in pyuria.

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Age/Gender	: 48Y 0M 0D /Female	Sample Collected On	: 07/Jun/2026 07:01AM	
Order Id	: 18603655290	Sample Received On	: 07/Jun/2026 05:50PM	
Referred By	: Self	Report Generated On	: 07/Jun/2026 07:01PM	
Customer Since	: 07/Jun/2026	Sample Temperature	: Maintained ✓	
Sample Type	: Whole Blood EDTA	Report Status	: Final Report	

DEPARTMENT OF HAEMATOLOGY

Healthians Summer Wellness Package Female FOR COUPLE COMBO

Test Name	Value	Unit	Bio. Ref Interval
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ERYTHROCYTE SEDIMENTATION RATE (ESR)

ESR	26	mm/1st hour	0-12
Method: Correlated Rouleaux Analysis			
Machine: Yumizen H550E			

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Order Id	: 18603655290	Sample Received On	: 07/Jun/2026 05:50PM	
Referred By	: Self	Report Generated On	: 07/Jun/2026 07:06PM	
Customer Since	: 07/Jun/2026	Sample Temperature	: Maintained ✓	
Sample Type	: Whole Blood EDTA	Report Status	: Final Report	

DEPARTMENT OF HAEMATOLOGY

Healthians Summer Wellness Package Female FOR COUPLE COMBO

Test Name	Value	Unit	Bio. Ref Interval
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Complete Blood Count Advanced

Haemoglobin (HB) Method: Spectrophotometry Machine: Yumizen H1500	12.5	g/dL	12.0-15.0
Total Leucocyte Count (TLC) Method: Impedance Machine: Yumizen H1500	3.7	10 ³ /uL	4.0-10.0
Hematocrit (PCV) Method: Calculated Machine: Yumizen H1500	39.0	%	36.0-46.0
Red Blood Cell Count (RBC) Method: Impedance Machine: Yumizen H1500	3.82	10 ⁶ /μl	3.80-4.80
Mean Corp Volume (MCV) Method: Derived from RBC Histogram Machine: Yumizen H1500	102.0	fL	83.0-101.0
Mean Corp Hb (MCH) Method: Calculated Machine: Yumizen H1500	32.8	pg	27.0-32.0
Mean Corp Hb Conc (MCHC) Method: Calculated Machine: Yumizen H1500	32.1	g/dL	31.5-34.5
RDW - CV Method: Derived from RBC Histogram Machine: Yumizen H1500	17.4	%	11.6-14.0
RDW - SD Method: Derived from RBC Histogram Machine: Yumizen H1500	64.30	fL	39.0-46.0
Mentzer Index Method: Calculated	26.70	Ratio	
RDWI Method: Calculated	464.61	Ratio	
Green and king index Method: Calculated	145	Ratio	

Differential Leucocyte Count

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DEPARTMENT OF HAEMATOLOGY

Healthians Summer Wellness Package Female FOR COUPLE COMBO

Test Name	Value	Unit	Bio. Ref Interval
Neutrophils Method: Flow-Cytometry DHSS Machine: Yumizen H1500	44.1	%	40 - 80
Lymphocytes Method: Flow-Cytometry DHSS Machine: Yumizen H1500	48.5	%	20-40
Monocytes Method: Flow-Cytometry DHSS Machine: Yumizen H1500	4.8	%	02 - 10
Eosinophils Method: Flow-Cytometry DHSS Machine: Yumizen H1500	2.2	%	01 - 06
Basophils Method: Impedance Machine: Yumizen H1500	0.4	%	00 - 02
Absolute Leucocyte Count			
Absolute Neutrophil Count (ANC) Method: Calculated	1.64	10 ³ /uL	2.0-7.0
Absolute Lymphocyte Count (ALC) Method: Calculated	1.81	10 ³ /uL	1.0-3.0
Absolute Monocyte Count Method: Calculated	0.18	10 ³ /uL	0.2-1.0
Absolute Eosinophil Count (AEC) Method: Calculated	0.08	10 ³ /uL	0.02-0.5
Absolute Basophil Count Method: Calculated	0.01	10 ³ /uL	0.02 - 0.10
Platelet Count(PLT) Method: Impedance Machine: Yumizen H1500	243	10 ³ /μl	150-410
MPV Method: Derived from PLT Histogram Machine: Yumizen H1500	10.9	fL	7 - 9
NLR (Neutrophil Lymphocyte Ratio) Method: Calculated	0.91	Ratio	
PLR (Platelet Lymphocyte Ratio)	134	Ratio	

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Age/Gender	: 48Y 0M 0D /Female	Sample Collected On	: 07/Jun/2026 07:01AM	
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Referred By	: Self	Report Generated On	: 07/Jun/2026 07:06PM	
Customer Since	: 07/Jun/2026	Sample Temperature	: Maintained ✓	
Sample Type	: Whole Blood EDTA	Report Status	: Final Report	

DEPARTMENT OF HAEMATOLOGY

Healthians Summer Wellness Package Female FOR COUPLE COMBO

Test Name	Value	Unit	Bio. Ref Interval
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Method: Calculated

The International Council for Standardization in Haematology (ICSH) recommends reporting of absolute counts of various WBC subsets for clinical decision making. This test has been performed on a fully automated 5 part differential cell counter which counts over 10,000 WBCs to derive differential counts. A complete blood count is a blood panel that gives information about the cells in a patient's blood, such as the cell count for each cell type and the concentrations of Hemoglobin and platelets. The cells that circulate in the bloodstream are generally divided into three types: white blood cells (leukocytes), red blood cells (erythrocytes), and platelets (thrombocytes). Abnormally high or low counts may be physiological or may indicate disease conditions, and hence need to be interpreted clinically.

The Mentzer index is used to differentiate iron deficiency anaemia beta thalassemia trait. If a CBC indicates microcytic anaemia, these are two of the most likely causes, making it necessary to distinguish between them.

If the quotient of the mean corpuscular volume divided by the red blood cell count is then 13, thalassemia is more likely. If the result is greater than 13, then iron deficiency anaemia is more likely.

Green and King Index used to differentiate IDA from thalassemia trait value >65 is likely to be Iron Deficiency Anemia and value <220 more likely to be Beta Thalassemia Trait.

Calculated parameters are either derived from Impedance measure, RBC pulse measurement, RBC/platelet histograms or formula derived.

The neutrophil to lymphocyte ratio (NLR) is considered to be a sensitive indicator of infection, inflammation and sepsis, and a robust prognostic marker of disease severity.

The platelet-to-lymphocyte ratio (PLR) is a marker revealing shifts in platelet & lymphocyte cell counts due to acute inflammatory and pro-thrombotic states.

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Referred By	: Self	Report Generated On	: 07/Jun/2026 07:18PM	
Customer Since	: 07/Jun/2026	Sample Temperature	: Maintained ✓	
Sample Type	: Serum	Report Status	: Final Report	

DEPARTMENT OF IMMUNOLOGY

Healthians Summer Wellness Package Female FOR COUPLE COMBO

Test Name	Value	Unit	Bio. Ref Interval
Follicle Stimulating Hormone, FSH			
Follicle Stimulating Hormone (FSH)	73.35	mIU/mL	Follicular Phase: 2.5-10.2
Method: CLIA			Mid Cycle Peak: 3.4-33.4
Machine: Siemens Atellica IM1300			Luteal Phase: 1.5-9.1
			Pregnant :- <0.3
			Post Menopausal: 23.0-116.3

FSH stimulates follicular growth and in conjunction with hLH, stimulates estrogen secretion and ovulation. Human FSH is secreted by the gonadotropic cells of the anterior lobe of the pituitary gland in response to gonadotropin releasing hormone (GnRH) from the medial basal hypothalamus. Abnormal FSH levels are interpreted with increased or decreased levels of other fertility hormones such as LH, estrogens, progesterone, and testosterone. Increased FSH levels are associated with menopause and primary ovarian hypofunction in females and primary hypogonadism in males. Decreased FSH levels are associated with primary ovarian hyperfunction in females and primary hypergonadism in males. Normal or decreased FSH levels are associated with polycystic ovary disease in females.

Luteinizing Hormone, LH

Luteinizing Hormone (LH)	32.690	mIU/ml	Mid Follicular Phase: 1.9-12.5
Method: CLIA			Mid Cycle Peak: 18.7-76.3
Machine: Siemens Atellica IM1300			Mid Luteal Phase: 0.5-16.9
			pregnant: <0.1-1.5
			Post Menopausal: 15.9-54.0
			contraceptive : 0.7-5.6

Human Luteinizing Hormone (LH) is a glycoprotein hormone secreted by the anterior pituitary gland. LH, FSH and other steroid hormones play important role in regulating the ovulation and ovarian functions during the menstrual cycle. Maturation of an ovarian follicle and its oocyte begins during the end of the preceding menstrual cycle. When FSH is released by the pituitary gland, the ovarian follicle undergoes rapid increases. This rapid increase is generally believed to trigger the rapid rise and peak of LH activity at mid-cycle, the LH surge. Ovulation begins twelve to twenty-four hours after the LH surge. The wall of the enlarged follicle ruptures and the mature ovum is extruded. Within two days the LH concentration returns to its baseline level. A concomitant rise in progesterone level initiates the luteal phase, which lasts approximately fourteen days. During this time, if the mature ovum is not fertilized, a new follicle enters the maturation route marking the beginning of the next menstrual cycle. In light of the characteristic fluctuation of LH levels during the menstrual cycle, rapid and sensitive detection of LH is useful in the diagnosis and treatment of infertility and in the detection of the LH surge to predict the time of ovulation. As the onset of the LH surge precedes ovulation by approximately thirty hours, its determination has been used successfully to time oocyte retrieval for in vitro fertilization and to assist in the timing of artificial insemination. INCREASED LEVELS are seen in Luteal phase of menstrual cycle, Primary hypogonadism, Gonadotropin secreting pituitary tumours, Menopause. DECREASED LEVELS are seen in Hypothalamic gonadotropin releasing hormone deficiency, Pituitary LH deficiency, Ectopic steroid hormone production, Gonadotropin releasing hormone analog treatment.



Dr. Preeti
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Consultant Biochemist
DMC - 97534, Healthians Labs



SIN No: E8267988

Patient Name	: Rashmi Sinha	Barcode	: E8267988	
Age/Gender	: 48Y 0M 0D /Female	Sample Collected On	: 07/Jun/2026 07:01AM	
Order Id	: 18603655290	Sample Received On	: 07/Jun/2026 05:49PM	
Referred By	: Self	Report Generated On	: 07/Jun/2026 07:18PM	
Customer Since	: 07/Jun/2026	Sample Temperature	: Maintained ✓	
Sample Type	: Serum	Report Status	: Final Report	

DEPARTMENT OF IMMUNOLOGY

Healthians Summer Wellness Package Female FOR COUPLE COMBO

Test Name	Value	Unit	Bio. Ref Interval
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CEA (Carcino Embryonic Antigen)

CARCINO EMBRYONIC ANTIGEN (CEA)	1.75	ng/mL	Non smokers < 2.50 Smokers < 5.00
Method: CLIA			
Machine: Siemens Atellica IM1300			

Clinical Significance: CEA is a glycoprotein involved in cell adhesion. It is normally produced in fetal gastrointestinal tissue, and its production stops before birth. Healthy adults normally have very low levels of CEA in their blood. Elevated levels may be associated with various malignant and non-malignant conditions.

1. Increased levels can be seen in:

- Malignant (including but not limited to)- Colorectal Carcinoma, Breast Cancer, Pancreatic Cancer, Lung Cancer, Medullary Thyroid Carcinoma, Ovarian/Cervical Carcinoma.
- Non-cancerous (Benign) conditions (including but not limited to)- Active Nonmalignant inflammatory diseases (especially of GI Tract) (Ulcerative Colitis, Regional Enteritis, Cholecystitis, Diverticulitis, Peptic Ulcers, Chronic Pancreatitis, Inflammatory Bowel Disease), Liver Disease (Alcoholic, Cirrhosis, Chronic Active Hepatitis, Obstructive Jaundice), Renal Disorder, Fibrocystic Disease of Breast, Smokers, Breast Cysts, Chronic Pulmonary Diseases, Mucinous Cystadenoma of the Ovary/Appendix, Lungs Issues, Ulcers, etc.

2. Decreased levels can be seen in:

- Effective therapy or post removal of the tumor

Note:

- The diagnostic efficiency of any tumor marker depends on a variety of factors such as its sensitivity, specificity, positive predictive value (PPV) and negative predictive value (NPV). A single tumor marker test result should not be used in isolation for the diagnosis/ confirmation of any suspected malignancy. It should be used in association with clinical correlation/ imaging studies/ further investigations, under the supervision of a clinician only
- A single tumor marker test result should not be used in isolation for the diagnosis/ confirmation of any suspected malignancy. It should be used in association with clinical correlation/ imaging studies/ further investigations, under the supervision of a clinician only
- Serial testing should be done to identify elevated levels caused due to transient physiological/ pathological factors
- Samples from patients receiving mouse monoclonal antibodies for diagnosis or therapy, may show falsely elevated or depressed values under specific conditions. Heterophilic antibodies in human serum, certain medications & non-specific protein binding may interfere with the test results
- Tumor marker test results are influenced by assay methods, reagent sensitivity and specificity, and should not be used interchangeably. If in the course of monitoring a patient, the assay method used is changed, additional testing should be carried out to

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Test Name	Value	Unit	Bio. Ref Interval
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confirm baseline values. All the results need to be followed up for correlation on similar platforms and methodologies

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Order Id	: 18603655290	Sample Received On	: 07/Jun/2026 05:49PM	
Referred By	: Self	Report Generated On	: 07/Jun/2026 08:04PM	
Customer Since	: 07/Jun/2026	Sample Temperature	: Maintained ✓	
Sample Type	: Serum	Report Status	: Final Report	

DEPARTMENT OF IMMUNOLOGY

Healthians Summer Wellness Package Female FOR COUPLE COMBO

Test Name	Value	Unit	Bio. Ref Interval
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Homocysteine

HOMOCYSTEINE	44.80	μmol/L	3.7-13.9
Method: CLIA			
Machine: Siemens Atellica IM1300			

Homocysteine is a thiol containing amino acid ,produced by intracellular demethylation of methionine to cysteine. Elevated Homocysteine has primary atherogenic and prothrombotic properties

Use

- Elevated Homocysteine levels may be used to exclude or confirm deficiencies of vitamin B 12 or folate
- Elevations in Homocysteine levels have also been used as an independent risk factor of coronary or cerebral vascular disease

Increased in

- Vitamin B12, Vitamin B 6 or folate deficiency
- Hypothyroidism
- Chronic renal failure
- Coronary heart disease

Decreased in

- Down syndrome
- Pregnancy
- Hyperthyroidism
- Early diabetes



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DEPARTMENT OF IMMUNOLOGY

Healthians Summer Wellness Package Female FOR COUPLE COMBO

Test Name	Value	Unit	Bio. Ref Interval
-----------	-------	------	-------------------

Vitamin B12

VITAMIN B12	161	pg/ml	211 - 912
-------------	-----	-------	-----------

Method: CLIA

Machine: Siemens Atellica IM1300

Interpretation

Vitamin B12 is a coenzyme that is involved in two very important metabolic functions vital to normal cell growth and DNA synthesis: 1) the synthesis of methionine from homocysteine and 2) the conversion of methyl malonyl CoA to succinyl CoA. Deficiency of this vitamin can lead to megaloblastic anemia and ultimately to severe neurological problems. It can also lead to macrocytic anemia, glossitis, peripheral neuropathy, weakness, hyperreflexia, ataxia, loss of proprioception, poor coordination, and affective behavioral changes. A significant increase in RBC mean corpuscular volume (MCV) may be an important indicator of vitamin B12 deficiency.

Patients taking vitamin B12 supplementation may have misleading results. A normal serum concentration of Vitamin B12 does not rule out tissue deficiency of vitamin B12. The most sensitive test for Vitamin B12 deficiency at the cellular level is the assay for methyl malonic acid (MMA). If clinical symptoms suggest deficiency, measurement of MMA and homocysteine should be considered, even if serum B12 concentrations are normal.

The Bio Ref Interval for Vitamin B12 has been changed on Beckman Coulter UniCel DxI800 with effect from 18.2.25.

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DEPARTMENT OF IMMUNOLOGY

Healthians Summer Wellness Package Female FOR COUPLE COMBO

Test Name	Value	Unit	Bio. Ref Interval
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Vitamin D, 25-Hydroxy

VITAMIN D (25 - OH VITAMIN D)	4.26	ng/ml	30 - 100
Method: CLIA			
Machine: Siemens Atellica IM1300			

VITAMIN D STATUS	VITAMIN D 25 HYDROXY (ng/mL), Adult	VITAMIN D 25 HYDROXY (ng/mL), Pediatric
DEFICIENCY	<20	<15
INSUFFICIENCY	20 - 30	15 - 20
SUFFICIENCY	30 - 100	20 - 100

Vitamin D is a steroid hormone known for its important role in regulating body levels of calcium and phosphorus and in the mineralization of bone

Uses:

- Diagnosis of Vitamin D deficiency
- Differential diagnosis of causes of rickets and Osteomalacia
- Monitoring Vitamin D replacement therapy
- Diagnosis of hypervitaminosis D

Increased in:

- Vitamin D intoxication
- Excessive exposure to sunlight

Decreased in:

- Malabsorption
- Steatorrhoea
- Dietary osteomalacia
- Thyrotoxicosis
- Coeliac disease
- Inflammatory bowel disease
- Rickets
- Pancreatic insufficiency



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DEPARTMENT OF IMMUNOLOGY

Healthians Summer Wellness Package Female FOR COUPLE COMBO

Test Name	Value	Unit	Bio. Ref Interval
-----------	-------	------	-------------------

Thyroid Profile (Total T3,T4, TSH)

Tri-Iodothyronine (T3, Total)	1.13	ng/ml	0.60-1.81
Method: CLIA			
Machine: Siemens Atellica IM1300			
Thyroxine (T4, Total)	6.50	ug/dl	3.2-12.6
Method: CLIA			
Machine: Siemens Atellica IM1300			
Thyroid Stimulating Hormone (TSH)-Ultrasensitive	18.416	μIU/ml	0.55-4.78
Method: CLIA			
Machine: Siemens Atellica IM1300			

Pregnancy interval	Bio Ref Range for TSH in uIU/ml (As per American Thyroid Association)
First trimester	0.1 - 2.5
Second trimester	0.2 - 3.0
Third trimester	0.3 - 3.0

Healthians

recommends that the following potential sources of variation should be considered while interpreting thyroid hormone results:

1. Thyroid hormones undergo rhythmic variation within the body this is called circadian variation in TSH secretion: Peak levels are seen between 2-4 AM. Minimum levels seen between 6-10 AM. This variation may be as much as 50% thus, influence of sampling time needs to be considered for clinical interpretation.
2. Circulating forms of T3 and T4 are mostly reversibly bound with Thyroxine binding globulins (TBG), and to a lesser extent with albumin and Thyroid binding Pre-Albumin. Thus the conditions in which TBG and protein levels alter such as chronic liver disorders, pregnancy, excess of estrogens, androgens, anabolic steroids and glucocorticoids may cause misleading total T3, total T4 and TSH interpretations.
3. Total T3 and T4 levels are seen to have physiological rise during pregnancy and in patients on steroid treatment.
4. T4 may be normal even in the presence of hyperthyroidism under the following conditions : T3 thyrotoxicosis, Hypoproteinemia related reduced binding, during intake of certain drugs (eg Phenytoin, Salicylates etc)
5. Neonates and infants have higher levels of T4 due to increased concentration of TBG
6. TSH levels may be normal in central hypothyroidism, recent rapid correction of hypothyroidism or hyperthyroidism, pregnancy, phenytoin therapy etc.
7. TSH values of <0.03 uIU/mL must be clinically correlated to evaluate the presence of a rare TSH variant in certain individuals which is undetectable by conventional methods.
8. Presence of Autoimmune disorders may lead to spurious results of thyroid hormones.
9. Various drugs influence the levels of thyroid hormones such as L-Dopa, Lithium, Glucocorticoids, Phenytoin etc.
10. Healthians recommends evaluation of unbound fractions, that is free T3 (fT3) and free T4 (fT4) for clinic-pathologic correlation, as these are the metabolically active forms.

*** End Of Report ***


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SIN No:E8267988



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Terms & Conditions:

- 1) Machine Data is available for last 7 days only. In case of manual testing & outsourced testing, machine data will not be available.
- 2) CBC parameters may vary when it is manually reviewed by the Pathologists.
- 3) **For Thyroid tests** - Circulating TSH shows a normal circadian rhythm with a peak between 11pm-5am and a nadir between 5pm-8pm. TSH values are also lowered after food when compared to fasting in a statistically significant manner. This variation is of the order of $\pm 50\%$, hence time of day and fasting status have influence on the reported TSH level.
- 4) **For Lipid profile** - Lipid and Lipoprotein concentrations vary during the normal course of daily activity. Also, certain drugs, diet and alcohol can have lasting effects on Triglyceride levels. To obtain best results for Lipid testing, a strict fasting of 10-12 hours with a light meal on the previous night is recommended.
- 5) Test results released pertain to the specimen submitted.
- 6) Test results are dependent on the quality of the sample received by the Lab.
- 7) The tests are carried out in the lab with the presumption that the specimen belongs to the patient named or identified in the bill/test request form/booking ID.
- 8) The reported results are for information and are subject to confirmation and interpretation by the referring doctor to co-relate clinically.
- 9) Test results may show interlaboratory variations.
- 10) Liability of Healthians for deficiency of services or other errors and omissions shall be limited to the fee paid by the patient for the relevant laboratory services.
- 11) This report is not subject to use for any medico-legal purposes.
- 12) Few of the tests might be outsourced to partner labs as and when required.
- 13) This report is not intended to replace but to lead by providing comprehensive information. It is recommended that you consult your doctor/physician for interpretation of results.
- 14) All reports might not be applicable for individuals less than 18, pregnant women or individuals suffering from diseases for which health test has not been performed or symptoms not diagnosed.
- 15) This report is based on preventive health test screening and is meant for a healthy lifestyle. It does not provide any recommendation for life threatening situations.
- 16) It is strongly recommended to take required precautions for allergic reactions or sensitivities.
- 17) Authorised partner labs as mentioned for certain tests are as below:
 - HL/PL/001- Metropolis Healthcare Ltd
 - HL/PL/002- Thyrocare technologies Limited
 - HL/PL/003- Lifecell International Pvt. Ltd. - Laboratory Services
 - HL/PL/004- Modern Diagnostic & Research Centre

ADVISORY

Health Advisory

Rashmi Sinha

Booking ID : 18603655290 | Sample Collection Date : 07/Jun/2026

26.37 Body Mass Index

5'1" Height (ft/in)

65 Weight (kgs.)



Physical Activity

No Data



Smoke

No Data



Food Preference

No Data



Alcohol

No Data



Medication

No Data



Family History

No Data



Blood Pressure

No Data



Pulse Rate

No Data



Waist (In Cm)

0cm



Hip Circumference (In Cm)

No Data



SPO2 Levels

No Data



Sugar Levels

No Data

Additional Remarks :

NA

SUGGESTED NUTRITION

SUGGESTED NUTRITION

Do's

- Have a balanced diet that includes whole grains, pulses, dairy, fruits, vegetables, nuts and healthy fats
- Include fruits like apples, berries and melons in your diet
- Have high protein, moderate fat diet with low fat milk, yoghurt or buttermilk
- Include calcium rich foods like milk, yoghurt, cheese and green, leafy vegetables
- Include Brazil nuts, sesame seeds, sunflower seeds
- Include whole grains in your diet like whole wheat bread and other products, brown rice or hand pounded rice, oats
- Include zinc rich foods like oysters, shellfish and chicken

Dont's

- Avoid flavoured and seasoned foods
- Decrease intake of colas and sugary drinks
- Avoid saturated fats, trans fats, oily and greasy foods like cakes, creamy or fried foods
- Avoid salty foods and pickles
- Limit sugar intake
- Avoid high cholesterol and calorie dense foods
- Avoid the use of oil and avoid sauces and dressings
- Reduce caffeine intake
- Avoid cruciferous foods like cauliflower, cabbage and spinach
- Avoid soy products like soymilk or tofu

SUGGESTED LIFESTYLE

SUGGESTED LIFESTYLE

Do's

- Stay active and maintain ideal weight
- Maintain ideal weight
- Have regular exposure to sunlight

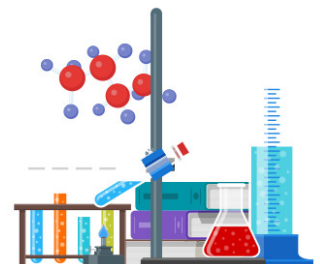
Dont's

- Avoid overexertion without having food or drink
- Avoid strenuous exercises
- Avoid smoking and alcohol
- Avoid overeating or calorie rich food
- Avoid having long gaps in meals or skipping meals
- Don't ignore your body signals and don't skip your regular health check-ups
- Avoid overworking or being stressed for long time

SUGGESTED FUTURE TESTS

SUGGESTED FUTURE TESTS

- Complete Hemogram - **Every 2 Month**
- Peripheral Smear Examination By Pathologist - **Every 2 Month**
- Vitamin B12 Cyanocobalamin - **Every 1 Month**
- Vitamin D Total-25 Hydroxy - **Every 1 Month**
- Folic Acid - **Every 1 Month**
- Electrocardiogram - **Every 1 Month**
- CPK, Total - **Every 1 Month**



HEALTH ADVISORY

Suggestions for Health & Well-being

Rashmi Sinha

Booking ID : 18603655290 | Sample Collection Date : 07/Jun/2026

PHYSICAL ACTIVITY

PHYSICAL ACTIVITY

Physical activities can vary from Regular walks (Brisk or normal), Jogging , Sports, Stretching, Yoga to light weight lifting etc. It is recommended to partake in physical activity at least 30 minutes a day for 3-4 days a week.

If regular workout is difficult, then we can adapt changes such as using stairs instead of lift/escalators and doing household work!



BALANCED DIET

A balanced diet is the key to healthy lifestyle. Include Whole grains, vegetables, whole fruits, nuts, seeds, beans, plant oils in your diet.

It is recommended to always have a high protein breakfast and a light dinner. Avoid items such as processed foods, potatoes and high calorie/sugar products. Don't forget to drink water regularly!

BALANCED DIET

STRESS MANAGEMENT

STRESS MANAGEMENT

Managing stress is an essential part of well-being. Some day to day changes can help such as having sufficient sleep (6-8 hours), indulging yourself in meditation, positive attitude towards lifestyle, using humor, traveling, talking to people whom you feel comfortable with and making time for hobbies by doing what you love to do.



BMI

BMI recommended range is 18.5 to 24.9. Your BMI is **26.37**, which is on a higher side.

Please follow recommended diet and maintain a healthy lifestyle and try to keep your BMI within the desired range. Keeping the right BMI for your body helps prevent many untimely diseases and goes a long way.

BMI CHART



SUGGESTED BMI

Supplement Suggestions

Rashmi Sinha

Booking ID : 18603655290 | Sample Collection Date : 07/Jun/2026

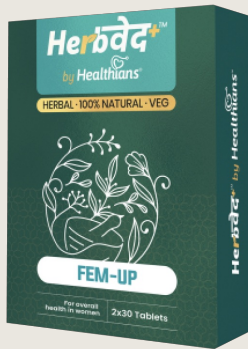
Your test report has indicated that you have certain deficiencies in your body which may hamper your health & wellbeing in the longer run.

In order to fulfill the gaps in nutrition and promote a healthier body we suggest you the following supplements mentioned below:

Deficiency/Out of Range Parameter(s)	Suggested Supplement
LH-Luteinizing Hormone	FEM-UP
TSH Ultra - Sensitive	THYRO FIX

To order, call 1800-572-000-4

Suggestions for Improving Deficiencies



FEM-UP

For Pink of Health in Women

Polycystic ovarian disease (PCOD) giving you sleepless nights? Let the power of Ayurveda become your shield against PCOD effects with FEM-UP. An all-natural supplement that helps in alleviating the debilitating impact of PCOD. Infused with the natural goodness of Ashwagandha, Shatavari, and Trikatu, FEM-UP helps reduce androgen levels and PCOD-induced facial hair, aids in effective weight management, and boosts ovarian health, among numerous other benefits.

Remember, ignoring PCOD can cause a lot of serious complications, including:

Type 2 diabetes | Sleep apnea | Endometrial cancer | Infertility | Miscarriage

Infused with the ages-proven goodness of all-natural ingredients, FEM-UP is the perfect supplement to promote overall women's health, correct irregular menses, and counter PCOD effects without having to worry about side effects. Sourced from nature's pharmacy of herbs, the ingredients in FEM-UP present the following benefits:

Ashwagandha

Improves sex drive. Provides relief from insomnia & fatigue

Shatavari

Helps restore PCOD-induced hormonal imbalance

Shankhpushpi

Helps improve digestion & reduce depression

Musli

Improves sexual performance & overall health

Brahmi

Reduces anxiety, depression & stress



THYRO FIX

Here's nature's way to improve your thyroid function.

THYRO-FIX is a scientifically formulated and clinically proven all-natural supplement that helps strike the optimum balance of your thyroid levels. Whether hyperthyroidism or hypothyroidism, this ayurvedic supplement keeps your thyroid balanced and optimally functioning. Take the all-natural road to a healthy thyroid with THYRO-FIX.

Be it hyperthyroidism or hypothyroidism, untreated thyroid conditions can cause serious health issues, such as:

• Cardiovascular Diseases | • Brittle Bones | • Eye Issues | • Infertility | • Mental Health Concerns

Infused with the ages-proven goodness of all-natural ingredients, THYRO-FIX is the perfect supplement to promote and maintain good thyroid health, without having to worry about side effects. Sourced from nature's own pharmacy of herbs, the ingredients in THYRO-FIX present the following benefits:

Arjun Tree Extract

Decreases thyroid levels in hyperthyroidism to maintain hormonal balance

Ashwagandha

Increases thyroid levels in hypothyroidism to maintain hormonal balance

Anantmoool

Anti-inflammatory & anti-oxidant properties reduce the symptoms of thyroid disorder

Asparagus

Regulates blood sugar levels & promotes heart health

Supplement Suggestions

Rashmi Sinha

Booking ID : 18603655290 | Sample Collection Date : 07/Jun/2026

Deficiency/Out of Range Parameter(s)	Suggested Supplement
Vitamin B12 Cyanocobalamin	VITAMIN B12
Vitamin D Total-25 Hydroxy	VITAMIN D3

To order, call 1800-572-000-4

Suggestions for Improving Deficiencies



VITAMIN B12

Make healthier blood the foundation of a healthier life!

Keep your blood clean and your nerve cells healthy with Vitamin B12. This essential supplement also protects against anemia, which causes persistent fatigue. Extracted from natural food sources, such as amla and beetroot, daily intake of Vitamin B12 can help in the formation of red blood cells.

A lack of vitamin B12 can cause dangerous health complications, such as:

- Memory Loss
- Vision Loss
- Ataxia or Loss of Physical Coordination
- Irreversible Neurological Problems
- Heart Diseases

Prevents Osteoporosis & Promotes Bone Health

Prevents Anemia & Promotes Red Blood Cell (RBC) Production

Alleviates Symptoms of Depression

Boosts Heart Health

Prevents Major Birth Defects



VITAMIN D3

Improve bone health with enhanced calcium absorption, the natural way

Make your muscles and bones stronger with VITAMIN D3. Sourced from natural substances, it helps in regulating the absorption of calcium and phosphorus, which help keep your bones strong and enhancing the normal immune system functioning. Vitamin D3 is an essential nutrient that's critical for normal growth and development of bones and teeth, as well as improved resistance against certain diseases.

Remember, a lack of vitamin D3 can cause dangerous health situations.

- Rickets (in children)
- Brittle Bones
- Osteoporosis
- Weakened Bones (in adults)

Strengthens Bones & Muscles

Protects Against Pneumonia & Acute Respiratory Infections

Helps in Reducing Depression

Boosts Heart Health

Aids in Kidney Disease Treatment

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About Healthians Labs

How we control Report Accuracy at Healthians



Quality Control

We follow Quality control to ensure both **precision & accuracy** of patient results.



Machine Data

We save patient's result values **directly from machines** ensuring no manipulations & no fake values.



QR Code

QR Code based authenticity check on all its reports



Calibration

We make use of calibrators to evaluate the **precision & accuracy** of measurement equipment.



Equipment

Our Labs are equipped with state-of-the-art instruments with **cutting edge technology** to provide faster & reliable results.



EQA

Our Labs participate in EQA & show proven accuracy by checking **laboratory performance** through external agency or facility.

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